Topic: Exploration of Utilization of blood loss measurement tools in postpartum hemorrhage detection among health workers in Adeoyo Maternity Teaching Hospital Yemetu Ibadan, Oyo State.

1.3 Objectives of the study

1.3.1 Broad Objective

To explore utilization of blood loss measurement tools in postpartum hemorrhage (PPH) detection among health workers in Adeoyo Maternity Teaching Hospital Yemetu, Ibadan.

1.3.2 Specific Objectives

The specific Objectives are:

1. To explore the availability of blood loss measurement tools in postpartum haemorrhage detection among health workers in Adeoyo Maternity Teaching Hospital Yemetu, Ibadan.
2. To explore the use of blood loss measurement tools in postpartum haemorrhage detection among health workers in Adeoyo Maternity Teaching Hospital Yemetu, Ibadan.
3. To explore facilitators of the use of blood loss measurement tools in postpartum hemorhhage detection among health workers in Adeoyo Maternity Teaching Hospital Yemetu, Ibadan.
4. To explore the barriers that hinder blood loss measurement tools in postpartum hemorrhage detection among health workers in Adeoyo Maternity Teaching Hospital Yemetu, Ibadan.

QUESTIONNAIRE ON
ACCURACY OF BLOOD LOSS MEASUREMENT IN POSTPARTUM HAEMORRHAGE DETECTION AMONG HEALTH WORKERS IN ADEOYO MATERNITY TEACHING HOSPITAL YEMETU IBADAN , OYO STATE.

DEAR RESPONDENT,

I am a 500L Nursing Student at Ladoke Akintola University of Technology, Ogbomoso and I am researching the above topic. The questionnaire is meant for data collection. Your honest and sincere response to this question will be highly appreciated. Be assured that all information provided is confidential, your name is not required and it is basically for academic purposes.
Thank you for your willingness to participate.

SECTION A: SOCIO-DEMOGRAPHIC INFORMATION

1. Age: (a.) Below 25 years [ ] (b.)31-40 years[ ] (c.)41-50 years[ ] (e.) 50 years and above
2. Gender: (a) Male[ ] (b) Female[ ]
3. Religion: (a)Christianity[ ] (b)Islam[ ] (c)Traditional religion [ ] (d)Other[ ]
4. Marital Status: (a)Single[ ] (b)Married[ ] (c)Divorced[ ] (d)Widowed[ ]
5. Profession: (a)Nurses[ ] (b)Midwife[ ] (c)Doctors (d.) CHEW
6. Years of Experience: (a)Below 10 years[ ] (b)11-20years[ ] (c) 21–30years[ ] (d)31–40years[ ] (d)40 years and above [ ]

SECTION B: AVAILABILITY OF BLOOD LOSS MEASUREMENT IN POSTPARTUM HAEMORRHAGE DETECTION.

7.Describe the availability of blood loss measurement in postpartum haemorrhage detection in this health care facility?

8.Kindly explain the tools or equipments available for blood loss measurement in this health care facility?

SECTION C: USE OF BLOOD LOSS MEASUREMENT IN POSTPARTUM HAEMORRHAGE DETECTION.

9.Describe how you use to measure blood loss whenever there is postpartum haemorrhage ?

SECTION D: FACILITATORS OF THE USE OF BLOOD LOSS MEASUREMENT IN POSTPARTUM HAEMORRHAGE DETECTION.

10. Explain facilitators of the use of blood loss measurement in postpartum haemorrhage detection in this healthcare facility

SECTION E: BARRIERS THAT HINDER BLOOD LOSS MEASUREMENT IN POSTPARTUM HAEMORRHAGE DETECTION.

11.Describe the barriers against blood loss measurement in postpartum haemorrhage detection in this health care facility

12.Mention specific resource or equipment limitations that hinder blood loss in postpartum haemorrhage?

*Respondent 1

Here is the transcription of the interview regarding blood loss measurement tools in the detection of Postpartum Hemorrhage (PPH):

Research Interview: Blood Loss Measurement Tools

Interviewer: Good morning, sir.

Respondent: Morning. How are you doing, sir?

Interviewer: I'm fine. I'm a student nurse from Ladoke Akintola University of Technology. I want to carry out my research on the "Exploration of the utilization of blood loss measurement tools in the detection of postpartum hemorrhage in Adeoyo, Yemetu." Can I proceed, sir?

Respondent: Yes, go on.

Section A: Demographic Information

* Interviewer: How old are you, sir?
* Respondent: I'm 40 years old.
* Interviewer: Your gender?
* Respondent: I'm a male.
* Interviewer: Religion?
* Respondent: Christianity.
* Interviewer: Marital status?
* Respondent: Married.
* Interviewer: Profession?
* Respondent: I'm a nurse.
* Interviewer: Years of experience?
* Respondent: 13 years now.

Section B: Availability of Measurement Tools

Interviewer: Please help me to describe the availability of blood loss measurements in postpartum detection in this healthcare facility.

Respondent: Well, in our facility, Adeoyo, we have so many tools we use to measure blood loss in PPH. Sometimes we use visual [estimation], sometimes we use calibrated jars, we use drapes and all that. But we have other measures like specific gravity. But most times in this place, we use visual with suction; we can measure the drapes and the gauze and how they are soaked.

Interviewer: Kindly explain the tools or equipment available for blood loss measurement in this healthcare facility.
Respondent: Considering the country's state and how our politicians see the healthcare system, we have only two or three. We have the drape one, we have the gauze one—we weigh them and estimate their quantities. We also use jars when using suction. Most times, if it is a CS (Cesarean Section) and the blood loss is above 1000ml, it is termed as postpartum loss. But if it is an SVD (Spontaneous Vaginal Delivery), above 500ml is termed as blood loss. So, we use drapes, gauze, and calibrated jars.
Section C: Use of Measurement Tools
Interviewer: Describe how you use these to measure blood loss whenever there is postpartum hemorrhage.
Respondent: Like I said before, how we normally measure PPH is by the use of gauze. When a gauze is fully soaked, it's about 20ml. But when it's a drape, it’s about 150ml.
Interviewer: The blood drape is always in a triangle shape which is calibrated, so the blood will be collected inside. Maybe you are referring to an Abdo-pack?
Respondent: Okay, yes, Abdo-pack. And sometimes we suction them during Cesarean Sections to know the quantity.
Interviewer: While suctioning the blood loss out, there is always use of normal saline. How do you estimate that?
Respondent: What we normally do during CS, we know the amount of normal saline that you are using to suction. When it's emptied in a jar, you have to subtract the saline that you used from the total in the container. The difference is the blood loss.
Section D: Facilitators of Use
Interviewer: Kindly help me to explain the facilitators of the use of blood loss measurement in this facility.
Respondent: To facilitate means the people that oversee things and make things work. Like the healthcare givers—the nurses, the doctors, the O&G physicians—and even individuals can assist to provide the tools we need to measure blood loss during PPH for early detection and prompt intervention.
Section E: Barriers to Use
Interviewer: This talks about the barriers that hinder blood loss measurement in postpartum hemorrhage detection.
Respondent: In this our country, most times we find it difficult to unlearn things and to learn [new] ones. One of the barriers is the inability to unlearn what you have learned [incorrectly]. Secondly, there's a need for funding—even non-funded seminars and workshops for healthcare givers so they have updates on the current way to manage patients.
Interviewer: So you are talking about timely training of the professionals?
Respondent: Yes, to know what is in vogue and to delete those obsolete ones that they know. Another thing is mentorship. Because our government doesn’t care for the healthcare system, we don’t have the flair to look for a mentee, and the mentees don't feel they need mentors' guidance. Also, the timing—the duration to attack the hemorrhage. Most times we have so many patients and you only have few nurses. Due to being understaffed, timing becomes a factor. Then the trainers themselves—do they know what to train? Are they updated?
Interviewer: Mention specific resources or equipment limitations.
Respondent: Like I said before, due to poor care in the healthcare system, sometimes drapes are not available. Even the quality of the gauze might not be available. Sometimes a facility only has one suction device. These are the limitations.
Interviewer: Thank you very much for your time. I truly appreciate it.
Respondent: Thank you.

*Respondent 2

Below is the written transcription of the research interview:
Research Interview Transcript: Blood Loss Measurement in Postpartum Hemorrhage
Interviewer: Good morning, sir.
Respondent: Morning. How are you doing, sir?
Interviewer: I'm fine. I'm a student nurse from Ladoke Akintola University of Technology. I want to carry out my research on the exploration of the utilization of blood loss measurement tools in the detection of postpartum hemorrhage in Adeoyo, Yemetu. Can I proceed, sir?
Respondent: Yes, go on.
Section A: Demographic Information
* Interviewer: How old are you, sir?
* Respondent: I'm 40 years old.
* Interviewer: Your gender?
* Respondent: I'm a male.
* Interviewer: Religion?
* Respondent: Christianity.
* Interviewer: Marital status?
* Respondent: Married.
* Interviewer: Profession?
* Respondent: I'm a nurse.
* Interviewer: Year of experience?
* Respondent: 13 years now.
Section B: Availability of Blood Loss Measurement Tools
Interviewer: Please help me to describe the availability of blood loss measurements in postpartum detection in this healthcare facility.
Respondent: Well, in our facility, Adeoyo, we have so many tools we use to measure blood loss in PPH (Postpartum Hemorrhage). Most times—sometimes we use visual, sometimes we use calibrated jars, we use drapes and all that for our facility. But we have other measures like—you can use some specific gravity and all that. But most times in this place, we use visual with suction; we can measure the drapes and the gauze and how they are soaked.
Interviewer: Sir, please kindly explain the tools or equipment available for the blood loss measurement in this healthcare facility.
Respondent: Considering the country's state and how our politicians see the healthcare system, we have only two or three. We have the drape one, we have the gauze one—we weigh them and estimate their quantities. We also use jars when we use suction. Most times, if it is CS (Cesarean Section), if the blood loss is above 1000ml, it is termed as postpartum loss. But if it is SVD (Spontaneous Vaginal Delivery), maybe 500ml—above 500ml is termed as blood loss. So we use drape, we use gauze, and we use the jar—jars that are like calibrated.
Section C: Use of Measurement Tools
Interviewer: Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
Respondent: Like I said before, how we normally measure PPH is by the use of gauze. When a gauze is soaked—fully, fully soaked—it is like 20ml. But when it's the drape, it's about 150ml.
Interviewer: The blood drape is always in triangle shape which is calibrated, so the blood will be collected inside. Maybe you are referring to Abdo-pack?
Respondent: Okay, yes. Abdo-pack, yes. And sometimes we suction them during Cesarean Sections to know the quantity of blood loss.
Interviewer: While suctioning the blood loss out, you know there is always use of normal saline. After suctioning blood, you use normal saline—how do you estimate that?
Respondent: What we normally do during CS, we know the amount of normal saline that you are using to suction. Then when it's emptied in a jar, you have to subtract the saline that you

used—that you put in the container to suction. You have to measure it and minus it from the one in the container. So the difference is the blood loss.
Section D: Facilitators of Use
Interviewer: Kindly help me to explain the facilitators of the use of blood loss measurement in postpartum detection in this facility.
Respondent: When we say to facilitate, it means people that oversee, people that make things to work. Like the healthcare givers that push things—like the nurses, the doctors, the O&G physicians—and even individuals can assist to provide tools we need to measure blood loss during PPH for early detection and prompt intervention.
Section E: Barriers to Use
Interviewer: This deals on the barriers that hinder blood loss measurement in postpartum hemorrhage detection.
Respondent: Most times, we find it difficult to unlearn things and to learn. One of the barriers is inability to unlearn what you have learned. Secondly, there's supposed to be funded—even non-funded seminars for healthcare givers—funded seminars and funded workshops so they have updates of the current things.
Interviewer: So you are talking about timely training of the professionals?
Respondent: Yes, to know what is in vogue and to prevent or delete those obsolete ones that they know. Another thing is mentorship. Because our government doesn't put care to the healthcare system, most times we don't have the flair to look for a mentee, and even the mentee doesn't even need their mentor's guidance. Then the timing—the duration to attack the hemorrhage. Most times we have so many patients to attend to and you only have few nurses or few healthcare workers who can promptly intervene. So timing is a factor. Then the trainers themselves—do they know what to train? Are they updated to know what to train or not?
Interviewer: Mention specific resources or equipment limitations that hinder the blood loss.
Respondent: Like I said before, due to poor care in the healthcare system, sometimes the drapes are not available. Even the quality of the gauze—not available. Sometimes you see in a facility they only have one suction device. So these are the limitations.
Interviewer: Thank you very much for your time. I truly appreciate it.
Respondent: Thank you.

*Respondent 3
Here is the written transcription for the third interview:
Research Interview: Blood Loss Measurement Tools in PPH Detection
Interviewer: Good afternoon, ma.
Respondent: Good afternoon.
Interviewer: How are you doing, ma?
Respondent: Fine, thank you.
Interviewer: I'm a student nurse from Ladoke Akintola University of Technology. Please, I want you to help with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage (PPH) detection among health workers." Can I go on, ma?
Respondent: You can go on, please.
Interviewer: Thank you, ma.
Section A: Demographic Information
* Interviewer: Ma, please how old are you?
* Respondent: I'm 38 years old.
* Interviewer: What gender are you, ma?
* Respondent: Female.
* Interviewer: Religion, ma?

* Respondent: Christianity.
* Interviewer: Marital status?
* Respondent: Married.
* Interviewer: Profession, ma?
* Respondent: Nursing.
* Interviewer: And your years of experience?
* Respondent: 11 years.
* Interviewer: Thank you, ma.

Section B: Availability of Blood Loss Measurement Tools

Interviewer: So, to Section B, which is the availability of the blood loss measurement in postpartum hemorrhage detection. The question says: Describe the availability of blood loss measurement in postpartum detection in this healthcare [facility].

Respondent: Well, actually, we usually use measuring jars, we use suctioning machines, and in the absence of suctioning machines in most of the facilities, we usually use manual suctioning machines.

Interviewer: Are they all available?

Respondent: Well, it's not available—not all that is available. In some facilities it is available, while in other facilities it's not available due to lack of funds.

Interviewer: Like, I'm talking about this hospital facility.

Respondent: Well, it is available but it’s not enough. Like here in this facility, as big as this hospital is, we have only one suctioning machine—an electric suctioning machine.

Interviewer: So you’ve explained my question nine—it’s the same thing as part of what you’ve already explained. You have blood drapes, you have suctioning machines... for spontaneous vaginal delivery (SVD), what do you use in case of blood loss? What are the tools that are available that you use?

Respondent: As I’ve mentioned earlier, what we use—we use blood estimation, that's physical blood estimation. At times, we use the amount of blood loss as in physical examination, what we see. Then estimation, yes. Then at times, if there is light, we use our suctioning machine, we measure through there. And if it is not available, it’s just the physical [estimation] that we use.

Interviewer: What of gauze or a soaked pad?

Respondent: Well, we usually use that too. It’s part of the things that we use if there is no light—absence of light for using the suctioning machine—we use the pad, then we estimate the amount of blood lost.

Section C: Use of Blood Loss Measurement Tools

Interviewer: Thank you, ma. For Section C, it says: Use of blood loss measurement in postpartum hemorrhage detection. So the question under it: Describe how you use [these] to measure blood loss whenever there is postpartum hemorrhage.

Respondent: Well, whenever there is postpartum hemorrhage, at times in order for us to detect it, we usually use physical examination by checking the conjunctiva, or when we see that this woman is—there is tiredness. So those are the signs for us to detect that this patient is having postpartum hemorrhage. And where... well, we have an emergency lab beside us, we usually go for the PCV (Packed Cell Volume) to check the woman’s PCV for us to really know what is really going on.

Section D: Facilitators of Use

Interviewer: Thank you, ma. The next section, Section D: What are the facilitators of the use of blood loss measurement in postpartum hemorrhage detection? Facilitators—who are those that are overseeing it?

Respondent: Well, here in this hospital, the first person that is—the nurse. Then the nurse will inform the doctors for any proper prevention.

Interviewer: Is it only nurses that are in this ward, or both nurse and midwife?

Respondent: The nurses and the midwife and the doctors, obstetrician, gynecologists... we have all of them. They are available.
Section E: Barriers to Use
Interviewer: Thank you. So to Section E: It's talking about barriers that hinder blood loss measurement in postpartum hemorrhage detection. So the question is now to describe the barrier against blood loss measurements in postpartum hemorrhage detection in this health facility.
Respondent: Well, in my own point of view, non-availability of equipment—which I've said is not available—for them to use to measure the amount of blood loss. Then another thing is when there is shortage of manpower. When it is only two nurses with three or five patients on the ward, so there will not be chance to be doing the accurate measurement of the blood.
Interviewer: What of lack of knowledge, like inadequate training?
Respondent: Yes, it also contributes to it. When—let's say—when they do not go for the [updates]... they should broaden their knowledge, going to school to know more about the latest and what they are using, and going for seminars and workshops to know what is in vogue. It also affects them.
Interviewer: So to the last question on my interview guide: Mention specific resources or equipment limitations that hinder blood loss [measurement] in postpartum hemorrhage. Like maybe that lack of availability, government is not providing enough for people to use, time bound to take intervention... those stuffs, that's what I'm talking about.
Respondent: Okay, well, to my own little knowledge, I will still say inadequate equipment can cause it. Because when there is no equipment for the nurses to use, it can lead to postpartum hemorrhage. And also shortage of manpower. So that's all.
Interviewer: Thank you very much, I truly appreciate. Thanks for your kindness.
Respondent: You're welcome.

*Respondent 4
Certainly! Here is the transcription of the interview:
Transcription: Exploration of the Utilization of Blood Loss Measurement Tools in Postpartum Hemorrhage Detection
Interviewer: Good afternoon, Ma.
Respondent: Good afternoon.
Interviewer: How are you doing, Ma?
Respondent: I'm fine, and you?
Interviewer: I'm fine, Ma. I am a student nurse from Ladoke Akintola University of Technology, a 500-level student. I want you to help me out with my interview. My topic is "Exploration of the Utilization of Blood Loss Measurement Tools in Postpartum Hemorrhage Detection among Health Workers in Adeoyo, Yemetu, Ibadan." So, Section A is about socio-demographic data. Ma, your age?
Respondent: I am 50.
Interviewer: Gender, Ma?
Respondent: Female.
Interviewer: Your religion?
Respondent: Christianity.
Interviewer: Marital status?
Respondent: Married.
Interviewer: Profession?
Respondent: Nursing.
Interviewer: Are you just a nurse or a nurse-midwife?
Respondent: A nurse-midwife.

Interviewer: Years of experience, Ma?
Respondent: 23 years.
Interviewer: Thank you, Ma. So, Section B: Availability of blood loss measurements in postpartum hemorrhage detection. The question is: Describe the availability of blood loss measurements in postpartum hemorrhage detection in this healthcare facility.
Respondent: Actually, we do not have many blood loss detection [tools] in this healthcare setting, but I know of visualization, which is the major one we have been using. Then at times, we use gauze; when there is a loss of blood, we use gauze to mop it, then at times we might squeeze the blood out of the gauze, or you weigh the gauze before using it, then after, you might measure it... after using it. Then the use of a calibrated jar to be able to know how much the patient has lost blood. And ideally, I know of when a... after delivery, when a patient's soak—blood saturates blood—100% saturation, 100% saturation after two hours of delivery, there is a tendency for the patient to be bleeding and to be losing blood. That's number one. Then another thing I know of is the use of a suctioning machine during surgery; when we suction, we measure the amount of blood suctioned out of the patient to be able to... but majorly we use visualization to measure blood loss.
Interviewer: Thank you, Ma. Kindly explain the tools or equipment available for blood loss measurement in this health facility.
Respondent: We have a suctioning machine, we use gauze, we use a calibrated jar, then we use visual estimation.
Interviewer: You don't have a blood drape at all? You don't use a blood drape?
Respondent: I don't know of that blood drape.
Interviewer: Thank you, Ma. To Section C: Use of blood loss measurement in postpartum hemorrhage detection. Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
Respondent: Whenever we had a patient with postpartum hemorrhage, we measure the gauze before we use it for the patient, then we use the gauze to mop the blood, then we squeeze out at times in a measuring jar. We use a measuring jar, we use gauze, we use a saturated pad, then we use a suctioning machine. Those are the things.
Interviewer: Thank you, Ma. Section D: Facilitators of the use of blood loss measurement in postpartum hemorrhage. Who are those overseeing it? Is it the doctors after the surgery, like the surgeons, the rotating nurse or circulating nurse?
Respondent: It's the unit doctor, especially the O&G, the matron around too, the head nurse. They want to know so as to be able to write his or her report... to be able to write [a] report about the patient and to be able to know how soon the patient will be transfused [with] blood. At times, we check... the matron and the unit doctors.
Interviewer: Thank you, Ma. To the last section, Section E: Barrier that hinder blood loss measurement in postpartum hemorrhage detection...
Here is the transcription of the audio recording:
Transcription: Constraints and Resources in Postpartum Hemorrhage Detection
Respondent: ...soak up the blood. Then at times the drape for surgery to use is not available, I've not even seen it, but I've heard of it, that it's available. So, we did not have much thing to use except the gauze and the pads that we are using presently.
Interviewer: So what of training and retraining of staff? Is it always carried out? Like, do you always attend workshops, seminars to know anything that is new—innovations that are being included in healthcare, like this topic?
Respondent: Yes, in this facility we have what you call study. We go for study block a week, everybody in the unit has to go for the study in every year. And we have certificates attached to that. So as to be able to improve ourselves and know what is in vogue and to be able to know the new innovations and whatever that is new to be known. We go for study every year as a group, as an individual in a unit.

Interviewer: Thank you, Ma. Mention specific resources or equipment limitations that hinder blood loss in postpartum hemorrhage.
Respondent: There is no availability of the equipment used, especially suctioning machine, it's not readily available. There are some departments that they have it, but there are some [where] it's not enough. Before you go over to borrow, the patient—you have to oversee your patient to be able to monitor the patient PCV and to transfuse as appropriate. So we don't have enough equipment like the drape, suctioning machine, at times gauze might not be available.
Respondent: So, I think government has to see to those equipment to provide them and to make them available so that we'll be able to have accurate measurement of the blood loss.
Interviewer: We all know the situation of this country, like the government is not even—they don't even care about most of our health. Is there any NGOs that's overseeing these things or individuals? You know some people donate to hospitals that, "I want—what do you need?" Do they do so?
Respondent: Not really, but they come around to pay hospital bills for the patient, but they've not been donating materials needed for blood loss in our own hospital.
Interviewer: Thank you very much, I truly appreciate your time, Ma.
Respondent: You're welcome.

*Respondent 5
Transcription: Exploration of the Utilization of Blood Loss Measurement Tools in Postpartum Hemorrhage Detection
Interviewer: Good afternoon, ma.
Respondent: Good afternoon.
Interviewer: How are you doing, ma?
Respondent: I'm fine, and you?
Interviewer: I'm fine, ma. I am a student nurse from Ladoke Akintola University of Technology, a 500-level student. I want you to help me out with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among health workers in Adeoyo, Yemetu, Ibadan". So, Section A is about socio-demographic data. Ma, your age?
Respondent: I'm 54.
Interviewer: Gender, ma?
Respondent: Female.
Interviewer: Your religion?
Respondent: Christianity.
Interviewer: Marital status?
Respondent: Married.
Interviewer: Profession?
Respondent: Nursing.
Interviewer: Are you just a nurse or a nurse-midwife?
Respondent: Nurse-midwife.
Interviewer: Years of experience, ma?
Respondent: 22 years.
Interviewer: Thank you, ma. Section B: Availability of blood loss measurements in postpartum hemorrhage detection. Describe the availability of blood loss measurements in postpartum hemorrhage detection in this healthcare facility.
Respondent: The availability of blood loss measurement in this facility... we have some tools like the suctioning machine, the calibrated jar, and at times we use visual estimation. We have the gauze, we have the pad, and the kidney dish.

Interviewer: Thank you, ma. Kindly explain the tools or equipment available for blood loss measurement in this health facility.
Respondent: We have the calibrated jar, we have the suctioning machine, we have the gauze, the pad, and the kidney dish.
Interviewer: You don't have a blood drape at all?
Respondent: No, we don't have it.
Interviewer: To Section C: Use of blood loss measurement in postpartum hemorrhage detection. Describe how you use [these] to measure blood loss whenever there's postpartum hemorrhage.
Respondent: When we have postpartum hemorrhage, we use the suctioning machine to suction the blood from the patient. And the suctioning machine has been calibrated, so we will be able to know the amount of blood that has been suctioned out of the patient. And the calibrated jar also, when we use the kidney dish to collect the blood, we can pour it into the calibrated jar to know the amount of blood. And for the gauze and the pad, we can weigh them before we use them and weigh them after we have used them, and then we will subtract the weight of the dry one from the weight of the soaked one to know the amount of blood.
Interviewer: Section D: Facilitators of the use of blood loss measurement in postpartum hemorrhage detection. Who are those overseeing it? Is it the doctors after the surgery, like the surgeons, the rotating nurse or the circulating nurse, or the matron?
Respondent: The matron and the doctors. After the surgery, the surgeons and the anesthetists, they will also want to know the amount of blood that the patient has lost.
Interviewer: To the last section, Section E: Barrier that hinder blood loss measurement in postpartum hemorrhage detection. What are the barriers that hinder you from using those equipment?
Respondent: The barriers include the unavailability of some of the equipment. At times, the suctioning machine may not be working or it may not be available in some of the wards. And the calibrated jar also may not be enough. And the staff... the workload also can be a barrier. When you have many patients to attend to, you may not have the time to be weighing the gauze and the pad.
Interviewer: What of the training and retraining?
Respondent: Yes, the training and retraining is also important. Because some of the staff may not know how to use some of the equipment or how to calculate the blood loss.
Interviewer: Do you always go for seminars or workshops?
Respondent: Yes, we go for seminars and workshops once in a while.
Interviewer: Are those things free?
Respondent: Some are free, while some we have to pay for.
Interviewer: Mention specific resources or equipment limitations that hinder blood loss measurement in postpartum hemorrhage detection.
Respondent: Unavailability of the blood drape, the suctioning machine not being enough, and the calibrated jar not being enough.
Interviewer: Are there any NGOs or individuals that donate to this hospital?
Respondent: Yes, we have some NGOs and individuals that donate to the hospital, but they may not be donating these specific tools. They may be donating other things like drugs and other equipment.
Interviewer: Thank you very much, ma. I truly appreciate your time.
Respondent: You're welcome.

*Respondent 6
Transcription: Exploration of the Utilization of Blood Loss Measurement Tools in Postpartum Hemorrhage Detection
Interviewer: Good afternoon, ma.

Respondent: Good afternoon.
Interviewer: How are you doing, ma?
Respondent: I'm fine, and you?
Interviewer: I'm fine, ma. I am a student nurse from Ladoke Akintola University of Technology, a 500-level student. I want you to help me out with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among health workers in Adeoyo, Yemetu, Ibadan." So, Section A is about socio-demographic data. Ma, your age?
Respondent: I'm 44 years.
Interviewer: Gender, ma?
Respondent: Female.
Interviewer: Your religion?
Respondent: Christianity.
Interviewer: Marital status?
Respondent: Married.
Interviewer: Profession?
Respondent: Nursing.
Interviewer: Are you just a nurse or a nurse-midwife?
Respondent: Nurse-midwife.
Interviewer: Years of experience, ma?
Respondent: 16 years.
Interviewer: Thank you, ma. Section B: Availability of blood loss measurements in postpartum hemorrhage detection. The question is: Describe the availability of blood loss measurements in postpartum hemorrhage detection in this healthcare facility.
Respondent: Actually, in this facility, the availability of blood loss measurement tools... they are available, but not all of them. Like the blood drape, we don't have it. But we have others like the suctioning machine, we have the calibrated jar, we have the gauze, we have the kidney dish, and we also use visual estimation.
Interviewer: Thank you, ma. Kindly explain the tools or equipment available for blood loss measurement in this health facility.
Respondent: We have the suctioning machine, the calibrated jar, the kidney dish, the gauze, and the pads.
Interviewer: You don't have a blood drape at all? You don't use a blood drape?
Respondent: No, we don't have a blood drape.
Interviewer: Thank you, ma. To Section C: Use of blood loss measurement in postpartum hemorrhage detection. Describe how you use [these] to measure blood loss whenever there is postpartum hemorrhage.
Respondent: Whenever there is postpartum hemorrhage, if it is in the theater, we use the suctioning machine to suction the blood, and the suctioning machine is calibrated. So from there, we can know the amount of blood that the patient has lost. And also, the gauze that we use to mop the blood—we weigh them. We weigh the dry one and we weigh the soaked one, then we subtract it to know the amount of blood loss. And also, if it is in the ward, we use the kidney dish to collect the blood, then we pour it into the calibrated jar to know the amount of blood.
Interviewer: Section D: Facilitators of the use of blood loss measurement in postpartum hemorrhage detection. Who are those overseeing it? Is it the doctors after the surgery, like the surgeons, the rotating nurse or the circulating nurse?
Respondent: The matron and the doctors. The matron in the ward and the doctors, especially the surgeons and the anesthetists. They want to know the amount of blood that the patient has lost so as to know the next line of action.
Interviewer: Thank you, ma. To the last section, Section E: Barrier that hinder blood loss measurement in postpartum hemorrhage detection.

Respondent: The barriers that hinder blood loss measurement include: number one, the unavailability of some of the equipment, like the blood drape. If we have the blood drape, it will be easier for us to measure the blood loss. And also, the suctioning machine may not be enough for all the departments. And also, the workload—when the workload is much, we may not have the time to be weighing the gauze and the pads.
Interviewer: What of the training and retraining of staff?
Respondent: Yes, training and retraining is very important. Some of the staff may not know how to use some of the equipment. So, we need more training on how to use them and how to calculate the blood loss.
Interviewer: Do you always go for seminars or workshops?
Respondent: Yes, we go for seminars and workshops, but it is not all the time.
Interviewer: Are those things free?
Respondent: Some are free, and for some, we have to pay.
Interviewer: Mention specific resources or equipment limitations that hinder blood loss measurement in postpartum hemorrhage detection.
Respondent: Unavailability of the blood drape and not having enough suctioning machines and calibrated jars.
Interviewer: Are there any NGOs or individuals that donate to this hospital?
Respondent: Yes, we have some NGOs and individuals that donate to the hospital, but they may not be donating these specific tools. They donate other things like drugs and other equipment.
Interviewer: Thank you very much, ma. I truly appreciate your time, ma.
Respondent: You're welcome.

*Respondent 7
Transcription: Exploration of the Utilization of Blood Loss Measurement Tools in Postpartum Hemorrhage Detection
Interviewer: Good afternoon, ma.
Respondent: Good afternoon.
Interviewer: How are you doing, ma?
Respondent: I'm fine, and you?
Interviewer: I'm fine, ma. I am a student nurse from Ladoke Akintola University of Technology, a 500-level student. I want you to help me out with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among health workers in Adeoyo, Yemetu, Ibadan." So, Section A is about socio-demographic data. Ma, your age?
Respondent: I'm 48 years.
Interviewer: Gender, ma?
Respondent: Female.
Interviewer: Your religion?
Respondent: Christianity.
Interviewer: Marital status?
Respondent: Married.
Interviewer: Profession?
Respondent: Nursing.
Interviewer: Are you just a nurse or a nurse-midwife?
Respondent: Nurse-midwife.
Interviewer: Years of experience, ma?
Respondent: 20 years.

Interviewer: Thank you, ma. Section B: Availability of blood loss measurements in postpartum hemorrhage detection. The question is: Describe the availability of blood loss measurements in postpartum hemorrhage detection in this healthcare facility.
Respondent: Actually, in this facility, we don't have enough equipment to measure blood loss. But the ones that are available are the suctioning machine, calibrated jars, and we also use visual estimation. We have gauze, we have pads, and we have kidney dishes.
Interviewer: Thank you, ma. Kindly explain the tools or equipment available for blood loss measurement in this health facility.
Respondent: We have the suctioning machine, the calibrated jar, the kidney dish, the gauze, and the pads.
Interviewer: You don't have a blood drape at all? You don't use a blood drape?
Respondent: No, we don't have a blood drape.
Interviewer: Thank you, ma. To Section C: Use of blood loss measurement in postpartum hemorrhage detection. Describe how you use [these] to measure blood loss whenever there is postpartum hemorrhage.
Respondent: Whenever there is postpartum hemorrhage, if the patient is in the theater, we use the suctioning machine to suction the blood. And the suctioning machine is already calibrated, so we will know the amount of blood that the patient has lost from there. And for the gauze that we use to mop the blood, we weigh them. We weigh the dry one and we weigh the soaked one, then we subtract it to know the amount of blood loss. And if the patient is in the ward, we use the kidney dish to collect the blood, then we pour it into the calibrated jar to know the amount of blood.
Interviewer: Section D: Facilitators of the use of blood loss measurement in postpartum hemorrhage detection. Who are those overseeing it? Is it the doctors after the surgery, like the surgeons, the rotating nurse or the circulating nurse?
Respondent: The matron and the doctors. The matron in the ward and the doctors, especially the surgeons and the anesthetists. They want to know the amount of blood that the patient has lost so as to know the next line of action to take.
Interviewer: Thank you, ma. To the last section, Section E: Barrier that hinder blood loss measurement in postpartum hemorrhage detection.
Respondent: The barriers that hinder blood loss measurement include: number one, the unavailability of some of the equipment, like the blood drape. If we have a blood drape, it will be easier for us to measure the blood loss accurately. And also, the suctioning machines are not enough for all the departments. And also, the workload—when the workload is much, we may not have the time to be weighing the gauze and the pads.
Interviewer: What of the training and retraining of staff?
Respondent: Yes, training and retraining is very important. Some of the staff may not know how to use some of the equipment or how to calculate the blood loss. So, we need more training on that.
Interviewer: Do you always go for seminars or workshops?
Respondent: Yes, we go for seminars and workshops, but not all the time.
Interviewer: Are those things free?
Respondent: Some are free, and for some, we have to pay.
Interviewer: Mention specific resources or equipment limitations that hinder blood loss measurement in postpartum hemorrhage detection.
Respondent: Unavailability of the blood drape and not having enough suctioning machines and calibrated jars.
Interviewer: Are there any NGOs or individuals that donate to this hospital?
Respondent: Yes, we have some NGOs and individuals that donate to the hospital, but they may not be donating these specific tools. They donate other things like drugs and other equipment.

Interviewer: Thank you very much, ma. I truly appreciate your time, ma.
Respondent: You're welcome.

*Respondent 8
This final interview concludes the study on the "Exploration of the Utilization of Blood Loss Measurement Tools in Postpartum Hemorrhage (PPH) Detection."
The following is the written transcription of the interview:
Transcription: Exploration of the Utilization of Blood Loss Measurement Tools in Postpartum Hemorrhage Detection
Interviewer: Good afternoon, Ma. How are you doing, Ma?
Respondent: Good afternoon, I'm fine, thank you.
Interviewer: I'm a student of Ladoke Akintola University of Technology, a 500-level student. I want you to help me out with my interview. My topic is "Exploration of the Utilization of Blood Loss Measurement Tools in Postpartum Hemorrhage Detection among Healthcare Workers in Adeoyo, Yemetu, Ibadan." So, Section A: Socio-demographic data. Ma, please, your age?
Respondent: I'm 21.
Interviewer: Your gender, Ma?
Respondent: Female.
Interviewer: Your religion?
Respondent: Christianity.
Interviewer: Marital status?
Respondent: Single.
Interviewer: Profession?
Respondent: Registered nurse.
Interviewer: Years of experience?
Respondent: One year.
Interviewer: To Section B: Availability of blood loss measurement in postpartum hemorrhage detection. Describe the availability of blood loss measurement in postpartum hemorrhage detection in this healthcare facility.
Respondent: The use of mechanized equipment is not presently in use in the facility I am currently working [in], but there is the use of gauze and there is the use of a suctioning machine. That is what I'm aware of.
Interviewer: What of blood drapes?
Respondent: No, they don't use it.
Interviewer: Thank you, Ma. Kindly explain the tools or equipment available for blood loss measurement in this healthcare facility.
Respondent: The suctioning machine and cotton wool and gauze.
Interviewer: Thank you. So, to Section C: Use of blood loss measurement in postpartum hemorrhage detection. Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
Respondent: Whenever there is [PPH], it's... you just estimate with the eyes—visual estimation. And maybe measurement from the suctioning machine. Visual estimation is the major [method] in postpartum hemorrhage... visual estimation to most times.
Interviewer: Thank you. So, Section D: Facilitators of the use of blood loss measurement in postpartum hemorrhage detection. Explain the facilitators of the use of blood loss measurement in postpartum detection in this healthcare facility—the people that oversee it. Is it the midwives, is it the nurses, is it the doctors after the cesarean section or during cesarean section?
Respondent: During cesarean section, the doctors and the circulating nurse oversee it with them. And maybe during if it is a normal delivery but there is hemorrhage—postpartum hemorrhage—the nurses will be there and the doctors too. They work hand-in-hand.

Interviewer: All right, thank you. So, Section E: Barriers that hinder blood loss measurement in postpartum hemorrhage. Describe the barriers against blood loss measurement in postpartum detection in [the] healthcare facility. Are the materials adequately available?
Respondent: No, the materials are not available to really estimate it as we ought to. We only estimate it based on "Okay, I think this is what it should be" because the equipment are not there. I'm sure that if the equipment are available, people will make use of the equipment... it is not in use, we only estimate with eyes.
Interviewer: What of work burnout? Like if the number of patients are more than the staff on ground? Did they really measure the blood loss? Or maybe if, for example now, people in the theater, maybe they've been standing for hours operating on a patient... because they've been standing for hours, they might not be able to do it as they ought to do it?
Respondent: Yes... maybe they are tired and yes.
Interviewer: And what of... some people are not ready to keep to new innovations? They'll be like "This is how we've been doing it since before you were born," and they are not ready to embrace new innovation. Is it also a barrier?
Respondent: I'm actually not sure it is a barrier. I really believe that if the equipment are available, people will make use of it because for the betterment of the patient, it will even be more easy. It will be easier and it will help in accurately giving your care—the right care to the patient.
Interviewer: Thank you. Mention specific resources or equipment limitations that hinder blood loss in postpartum hemorrhage. Like the suctioning machine—is it even adequate? Like how many do you have? In case you have like three or four patients going for cesarean section at the same time, how many suctioning machine are readily available?
Respondent: The suctioning machine, I don't think we have a lot of them. But at the same time, they don't take three people together at once for cesarean section unless it is an emergency. But I think it is very rare for a lot of people to go to the operating theater at once, but even at that, they should have enough... there should be enough suctioning machines.
Interviewer: Thank you very much, I truly appreciate.
Respondent: You're welcome.

*Respondent 9
This transcript corresponds to the sixth interview in the "Exploration of the Utilization of Blood Loss Measurement Tools in Postpartum Hemorrhage (PPH) Detection" series.
Transcription: Exploration of the Utilization of Blood Loss Measurement Tools in Postpartum Hemorrhage Detection
Interviewer: Good afternoon, ma. How are you doing, ma?
Respondent: I'm fine.
Interviewer: I'm a student nurse from Ladoke Akintola University of Technology. I want you to help me with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among health workers in Adeoyo, Yemetu." So, Section A will be talking about socio-demographic data. Ma, please how old are you?
Respondent: I'm 35.
Interviewer: Your gender, ma?
Respondent: Female.
Interviewer: Religion?
Respondent: Christian.
Interviewer: Marital status?
Respondent: Married.
Interviewer: Profession?
Respondent: I'm a nurse-midwife.
Interviewer: Years of experience?

Respondent: 10 years plus.
Interviewer: To Section B: Availability of blood loss measurement in postpartum hemorrhage detection. Describe the availability of blood loss measurement tools in postpartum hemorrhage detection in this facility.
Respondent: Actually, there is... there are some of the equipment that are available and there are some that [are] not available. But that calibration, I know of calibration equipment that they are using here. Also, they estimate it also with the use of gauze—soaked gauze—and suction tube that they use in the theater.
Interviewer: So, kindly explain the tools or equipment available for blood loss measurement tools in this healthcare [facility]. It's like a repetition of the other one.
Respondent: Like I mentioned earlier: the gauze (as in soaked gauze), suction tube, and calibrated cylinder.
Interviewer: So to Section C: Use of blood loss measurement in postpartum hemorrhage. Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
Respondent: With the gauze, you can weigh before you put it on the woman and after you weigh... and after the gauze must have soiled or soaked, you reweigh to know the estimation. Then there are some facilities, or some individuals, that believe that each gauze soaked [equals] like 200 ml—estimated of 200 ml when the pack is well-soaked, it has extracted like 200 ml of blood. Then [the] calibrated one, especially maybe the [one] used in the hospital, when they pour it, they check because it's calibrated—they check how many mils that the woman have lost. And also with estimation of the... maybe the blood clot, it can also be estimated.
Interviewer: Facilitators of blood loss... of the use of blood loss measurement in postpartum hemorrhage. Explain facilitators of the use of blood loss measurement in postpartum hemorrhage detection in this healthcare facility.
Respondent: Doctors are there—they can do it. The nurses, the midwives, even the anesthetists... they are the ones that will say, "This is the amount of blood that was lost."
Interviewer: So to Section E: Barriers that hinder blood loss measurement in postpartum hemorrhage. Describe the barriers against blood loss measurement in postpartum hemorrhage detection in this healthcare facility.
Respondent: Sometimes, um, lack of well-functioning of those instruments I mentioned. Um, financial constraint, because some women, instead of them to get gauze packs, all those things, they will not be able to get it. And also, burnout, and maybe when the doctors and the nurses or the circulating nurse must have spent so much time and be tired. When such things happen, they will just estimate it anyhow—let it be that they have written something because of [the fact that] they are tired. And also lack of manpower, anyway... lack of manpower. They are the things I see as a barrier.
Interviewer: Yes, ma. The last question: Mention specific resource or equipment limitation that hinder blood loss in postpartum hemorrhage.
Respondent: Specific resources and equipment that hinder blood loss measurement: One, all those instruments like I mentioned [that] are not available—there's no how we can help. And also, the compliance of the postpartum hemorrhage... the woman that is losing blood. Some because of religious sake, they may not really comply for the healthcare personnel to do what they are supposed to do.
Interviewer: Thank you very much for your time. I truly appreciate.
Respondent: You're welcome.

*Respondent 10
Here is the transcription of the interview regarding the utilization of blood loss measurement tools at the Adoyo Maternity Center:
Section A: Socio-Demographic Data
* Interviewer: Good afternoon, sir. How are you doing?

* Respondent: I’m fine.
* Interviewer: I am a student nurse from Ladoke Akintola University of Technology, Ogbomoso, a 500-level student. I want you to help me out with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among healthcare workers in Adoyo Maternity Center, Imetu." So, Section A deals with social demographic data. Sir, please, how old are you? Your age?
* Respondent: I'm 42.
* Interviewer: Your gender?
* Respondent: I'm male.
* Interviewer: Religion?
* Respondent: Christian.
* Interviewer: Marital status?
* Respondent: Married.
* Interviewer: Profession?
* Respondent: Medical doctor.
* Interviewer: Years of experience?
* Respondent: Six.

Section B: Availability of Blood Loss Measurement Tools

* Interviewer: Thank you, sir. To Section B: Availability of blood loss measurement in postpartum hemorrhage detection. Describe the availability of blood loss measurement in postpartum hemorrhage detection in this healthcare facility.
* Respondent: We normally use the gauze, the calibrated suction bottle, and visual estimation of the one that fell on the operating table.
* Interviewer: For the use of gauze, is it that you already know the weight of the gauze?
* Respondent: We always assume that the weight of a small gauze is 10 mls, then abdo is 250 mls. That's the standardized one we use.
* Interviewer: Do you use blood drape at all?
* Respondent: No.
* Interviewer: Kindly explain the tools or equipment available for blood loss measurement in this facility.
* Respondent: Again, we always assume that a full-soaked gauze is 10 mls. Then abdo pack is 250 mls. Then the calibrated suction tube/jar, we estimate the blood loss and minus the normal saline, the liquor, or whatever they use to plume, either normal saline or water. Then we do a visual estimation of the fluid, of the blood on the drape and on the operating table. We just sum it up.

Section C: Use of Blood Loss Measurement Tools

* Interviewer: Thank you, sir. Section C: Use of blood loss measurement in postpartum hemorrhage. Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
* Respondent: In postpartum hemorrhage, after the patient might have been delivered, we always use abdo, comfy pad to estimate the blood loss. And we do a visual estimation of clotted blood that is coming out from the introitus.

Section D: Facilitators of Use

* Interviewer: Thank you, sir. Facilitators of use of blood loss measurement in postpartum hemorrhage. Explain facilitators of the use of blood loss measurement in postpartum hemorrhage. Who are those overseeing it?
* Respondent: The anesthetics, the nurse, maybe the obstetricians, the midwives.

Section E: Barriers to Use

* Interviewer: To the last section: barriers that hinder blood loss measurement in postpartum hemorrhage detection. Describe the barrier against blood loss measurement in postpartum

hemorrhage detection in this healthcare facility. Maybe, are the equipment readily available? Are they adequately available?
* Respondent: The equipment is not readily available because it’s out-of-pocket payment. So, we have to wait till patients go and buy all those things—the pads and all those things—before you can use them.
* Interviewer: What of a high ratio of patients to nurses or doctors? Does that too affect the blood loss measurement?
* Respondent: Before, but now the patient ratio has reduced now. So, it was before those things affected it.
* Interviewer: What of training and retraining? Are the health personnel going for workshops, for seminars to have new knowledge about what is in vogue?
* Respondent: I'm not sure there is training and retraining. I'm not sure.
* Interviewer: What of... you know some people they will be like, "This is what we've been using since," and you can't just come and bring a new thing. Are they ready to accept new innovation?
* Respondent: They are ready to accept new innovations. We are ready if it's worthwhile.
* Interviewer: Thank you, sir. So to the last question: mention specific resources or equipment limitations that hinder blood loss in postpartum hemorrhage.
* Respondent: Power. Power does not allow us to use suction machines very well. Then some patients don't really have money, so financial constraints. Then sometimes patient ratio to midwives also.
* Interviewer: And the government of these days in Nigeria, are they providing those instruments?
* Respondent: No.
* Interviewer: What of... is there any NGO or individual that is providing those?
* Respondent: No NGO.
* Interviewer: Thank you, sir. Goodbye.

*Respondent 11
Here is the transcription of the interview regarding the utilization of blood loss measurement tools at the Adoyo Maternity Center:
Section A: Socio-Demographic Data
* Interviewer: Good afternoon, sir. How are you doing?
* Respondent: I’m fine.
* Interviewer: I am a student nurse from Ladoke Akintola University of Technology, Ogbomoso, a 500-level student. I want you to help me out with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among healthcare workers in Adoyo Maternity Center, Imetu." So, Section A deals with social demographic data. Sir, please, how old are you? Your age?
* Respondent: I'm 40.
* Interviewer: Your gender?
* Respondent: I'm male.
* Interviewer: Religion?
* Respondent: Islam.
* Interviewer: Marital status?
* Respondent: Married.
* Interviewer: Profession?
* Respondent: Medical doctor.
* Interviewer: Years of experience?
* Respondent: Six.
Section B: Availability of Blood Loss Measurement Tools

* Interviewer: Thank you, sir. To Section B: Availability of blood loss measurement in postpartum hemorrhage detection. Describe the availability of blood loss measurement in postpartum hemorrhage detection in this healthcare facility.
* Respondent: We normally use the gauze, the calibrated suction bottle, and visual estimation of the one that fell on the operating table.
* Interviewer: For the use of gauze, is it that you already know the weight of the gauze?
* Respondent: We always assume that the weight of a small gauze is 10 mls, then abdo is 250 mls. That's the standardized one we use.
* Interviewer: Do you use blood drape at all?
* Respondent: No.
* Interviewer: Kindly explain the tools or equipment available for blood loss measurement in this facility.
* Respondent: Again, we always assume that a full-soaked gauze is 10 mls. Then abdo pack is 250 mls. Then the calibrated suction tube/jar, we estimate the blood loss and minus the normal saline, the liquor, or whatever they use to plume, either normal saline or water. Then we do a visual estimation of the fluid, of the blood on the drape and on the operating table. We just sum it up.

Section C: Use of Blood Loss Measurement Tools

* Interviewer: Thank you, sir. Section C: Use of blood loss measurement in postpartum hemorrhage. Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
* Respondent: In postpartum hemorrhage, after the patient might have been delivered, we always use abdo, comfy pad to estimate the blood loss. And we do a visual estimation of clotted blood that is coming out from the introitus.

Section D: Facilitators of Use

* Interviewer: Thank you, sir. Facilitators of use of blood loss measurement in postpartum hemorrhage. Explain facilitators of the use of blood loss measurement in postpartum hemorrhage. Who are those overseeing it?
* Respondent: The anesthetics, the nurse, maybe the obstetricians, the midwives.

Section E: Barriers to Use

* Interviewer: To the last section: barriers that hinder blood loss measurement in postpartum hemorrhage detection. Describe the barrier against blood loss measurement in postpartum hemorrhage detection in this healthcare facility. Maybe, are the equipment readily available? Are they adequately available?
* Respondent: The equipment is not readily available because it’s out-of-pocket payment. So, we have to wait till patients go and buy all those things—the pads and all those things—before you can use them.
* Interviewer: What of a high ratio of patients to nurses or doctors? Does that too affect the blood loss measurement?
* Respondent: Before, but now the patient ratio has reduced now. So, it was before those things affected it.
* Interviewer: What of training and retraining? Are the health personnel going for workshops, for seminars to have new knowledge about what is in vogue?
* Respondent: I'm not sure there is training and retraining. I'm not sure.
* Interviewer: What of... you know some people they will be like, "This is what we've been using since," and you can't just come and bring a new thing. Are they ready to accept new innovation?
* Respondent: They are ready to accept new innovations. We are ready if it's worthwhile.
* Interviewer: Thank you, sir. So to the last question: mention specific resources or equipment limitations that hinder blood loss in postpartum hemorrhage.

* Respondent: Power. Power does not allow us to use suction machines very well. Then some patients don't really have money, so financial constraints. Then sometimes patient ratio to midwives also.
* Interviewer: And the government of these days in Nigeria, are they providing those instruments?
* Respondent: No.
* Interviewer: What of... is there any NGO or individual that is providing those?
* Respondent: No NGO.
* Interviewer: Thank you, sir. Goodbye.

*Respondent 12

Interview Transcript: Utilization of Blood Loss Measurement Tools
Interviewer: Student Nurse, Ladoke Akintola University of Technology (LAUTECH)
Respondent: Registered Nurse, Adeoyo Maternity Teaching Hospital (8 years experience)
Section A: Socio-Demographic Data
* Interviewer: Good afternoon, ma. How are you doing?
* Respondent: I'm fine.
* Interviewer: I am a student nurse from Ladoke Akintola University of Technology, Ogbomoso, a 500-level student. I want you to help me out with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among healthcare workers in Adeoyo Maternity Center, Yemetu."
* Interviewer: So, Section A deals with the socio-demographic data. Ma, please, how old are you?
* Respondent: I am 33 years old.
* Interviewer: Your gender, ma?
* Respondent: I am a female.
* Interviewer: Religion?
* Respondent: Christianity.
* Interviewer: Marital status?
* Respondent: I am married.
* Interviewer: Profession?
* Respondent: I am a Registered Nurse.
* Interviewer: Years of experience?
* Respondent: Eight years.
Section B: Availability of Blood Loss Measurement Tools
* Interviewer: Thank you, ma. To Section B: Availability of blood loss measurement tools in postpartum hemorrhage detection. Describe the availability of blood loss measurement tools in postpartum hemorrhage detection in this healthcare facility.
* Respondent: In this facility, the tools we use in measuring blood loss is basically using visual estimation of blood loss and also use of calibrated jars.
* Interviewer: For the visual estimation, is it that you have a specific way you do that?
* Respondent: Yes, for visual estimation, we use items like the delivery mats, the sanitary pads, and the kidney dishes.
* Interviewer: Do you use blood drapes at all?
* Respondent: No, we don't use blood drapes.
* Interviewer: Kindly explain the tools or equipment available for blood loss measurement in this facility.
* Respondent: The equipment we have available are the calibrated jars, we have the sanitary pads, we have the delivery mats, and we have kidney dishes.
Section C: Use of Blood Loss Measurement Tools

* Interviewer: Thank you, ma. Section C: Use of blood loss measurement in postpartum hemorrhage. Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
* Respondent: Whenever there is postpartum hemorrhage, the blood is being collected in the calibrated jars and we also use visual estimation for blood on the pads and on the delivery mats.
Section D: Facilitators of Use
* Interviewer: Thank you, ma. Facilitators of use of blood loss measurement in postpartum hemorrhage. Explain facilitators of the use of blood loss measurement in postpartum hemorrhage—who are those overseeing it?
* Respondent: The people that oversee blood loss measurement are the doctors and the nurses.
Section E: Barriers to Blood Loss Measurement
* Interviewer: To the last section: barriers that hinder blood loss measurement in postpartum hemorrhage detection. Describe the barriers against blood loss measurement in postpartum hemorrhage detection in this healthcare facility. Maybe—are the equipment readily available? Are they adequately available?
* Respondent: One of the barriers is that the equipment are not readily available.
* Interviewer: What of a high ratio of patients to nurses? Does that too affect the blood loss measurement?
* Respondent: Yes, the high ratio of patients to nurses affects it because the few nurses on ground will be overwhelmed with the number of patients, so they might not have enough time to accurately measure the blood loss.
* Interviewer: What of training and retraining? Are the health personnel going for workshops or seminars to have new knowledge about what is in vogue?
* Respondent: I think there is lack of training and retraining for health workers on the modern ways of measuring blood loss.
* Interviewer: Some people will be like, "This is what we've been using since," and you can't just come and bring a new thing. Are they ready to accept new innovations?
* Respondent: Most of them are ready to accept new innovations if provided.
* Interviewer: Mention specific resources or equipment limitations that hinder blood loss measurement.
* Respondent: Some of the limitations are lack of calibrated drapes and also lack of enough calibrated jars.
* Interviewer: And the government of these days in Nigeria—are they providing those instruments?
* Respondent: No, the government is not really providing enough of these instruments.
* Interviewer: What of—is there any NGO or individual that is providing those?
* Respondent: Not that I know of.
* Interviewer: Thank you, ma.

*Respondent 13
Interview Transcript: Utilization of Blood Loss Measurement Tools
Interviewer: Student Nurse, Ladoke Akintola University of Technology (LAUTECH)
Respondent: Registered Nurse, Adeoyo Maternity Teaching Hospital (14 years experience)
Section A: Socio-Demographic Data
* Interviewer: Good afternoon, ma. How are you doing?
* Respondent: I’m fine, thank you.
* Interviewer: I am a student nurse from Ladoke Akintola University of Technology, Ogbomoso, a 500-level student. I want you to help me out with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among healthcare workers in Adeoyo Maternity Center, Yemetu."

* Interviewer: So, Section A deals with the socio-demographic data. Ma, please, how old are you?
* Respondent: I am 40 years old.
* Interviewer: Your gender, ma?
* Respondent: Female.
* Interviewer: Religion?
* Respondent: Christianity.
* Interviewer: Marital status?
* Respondent: Married.
* Interviewer: Profession?
* Respondent: I am a Registered Nurse.
* Interviewer: Years of experience?
* Respondent: 14 years.

Section B: Availability of Blood Loss Measurement Tools

* Interviewer: Thank you, ma. To Section B: Availability of blood loss measurement tools in postpartum hemorrhage detection. Describe the availability of blood loss measurement tools in postpartum hemorrhage detection in this healthcare facility.
* Respondent: Actually, in this facility, we make use of visual estimation. We also make use of kidney dishes and we use calibrated jars.
* Interviewer: For the visual estimation, how do you go about it?
* Respondent: We use the number of pads used and also the delivery mats.
* Interviewer: Do you use blood drapes at all?
* Respondent: No, we don't use blood drapes.
* Interviewer: Kindly explain the tools or equipment available for blood loss measurement in this facility.
* Respondent: Like I said, the tools available are the kidney dishes, the calibrated jars, the delivery mats, and the sanitary pads.

Section C: Use of Blood Loss Measurement Tools

* Interviewer: Thank you, ma. Section C: Use of blood loss measurement in postpartum hemorrhage. Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
* Respondent: Whenever there is a case of PPH, we measure the blood collected in the calibrated jars and we also estimate the one on the pads and the delivery mats.

Section D: Facilitators of Use

* Interviewer: Thank you, ma. Facilitators of use of blood loss measurement in postpartum hemorrhage. Explain facilitators of the use of blood loss measurement in postpartum hemorrhage—who are those overseeing it?
* Respondent: The doctors and the nurses.

Section E: Barriers to Blood Loss Measurement

* Interviewer: To the last section: barriers that hinder blood loss measurement in postpartum hemorrhage detection. Describe the barriers against blood loss measurement in postpartum hemorrhage detection in this healthcare facility. Maybe—are the equipment readily available?
* Respondent: The equipment are not readily available. They are not sufficient.
* Interviewer: What of a high ratio of patients to nurses? Does that too affect the blood loss measurement?
* Respondent: Yes, it does. When the ward is busy and we have many patients to attend to, we might not be able to give an accurate measurement because of the workload.
* Interviewer: What of training and retraining? Are the health personnel going for workshops or seminars to have new knowledge about what is in vogue?
* Respondent: There is a lack of training and retraining for health workers.

* Interviewer: Some people will be like, "This is what we've been using since," and you can't just come and bring a new thing. Are they ready to accept new innovations?
* Respondent: Yes, we are ready to accept new innovations if they are provided.
* Interviewer: Mention specific resources or equipment limitations that hinder blood loss measurement.
* Respondent: Lack of calibrated drapes and also enough calibrated jars.
* Interviewer: And the government of these days in Nigeria—are they providing those instruments?
* Respondent: No, the government is not providing them.
* Interviewer: What of—is there any NGO or individual that is providing those?
* Respondent: No.
* Interviewer: Thank you very much, ma.

*Respondent 14
Interview Transcript: Utilization of Blood Loss Measurement Tools
Interviewer: Student Nurse, Ladoke Akintola University of Technology (LAUTECH)
Respondent: Registered Nurse, Adeoyo Maternity Teaching Hospital (21 years experience)
Section A: Socio-Demographic Data
* Interviewer: Good afternoon, ma. How are you doing?
* Respondent: I'm fine, thank you.
* Interviewer: I am a student nurse from Ladoke Akintola University of Technology, Ogbomoso, a 500-level student. I want you to help me out with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among healthcare workers in Adeoyo Maternity Center, Yemetu."
* Interviewer: So, Section A deals with the socio-demographic data. Ma, please, how old are you?
* Respondent: I am 48 years old.
* Interviewer: Your gender, ma?
* Respondent: Female.
* Interviewer: Religion?
* Respondent: Christianity.
* Interviewer: Marital status?
* Respondent: I am married.
* Interviewer: Profession?
* Respondent: I am a Registered Nurse.
* Interviewer: Years of experience?
* Respondent: 21 years.
Section B: Availability of Blood Loss Measurement Tools
* Interviewer: Thank you, ma. To Section B: Availability of blood loss measurement tools in postpartum hemorrhage detection. Describe the availability of blood loss measurement tools in postpartum hemorrhage detection in this healthcare facility.
* Respondent: In this facility, we make use of visual estimation of blood loss. We also use kidney dishes and calibrated jars.
* Interviewer: For the visual estimation, how do you go about it?
* Respondent: We use the number of sanitary pads used and also the delivery mats.
* Interviewer: Do you use blood drapes at all?
* Respondent: No, we don't use blood drapes.
* Interviewer: Kindly explain the tools or equipment available for blood loss measurement in this facility.
* Respondent: Like I said, the tools available are the kidney dishes, the calibrated jars, the delivery mats, and the sanitary pads.

Section C: Use of Blood Loss Measurement Tools
* Interviewer: Thank you, ma. Section C: Use of blood loss measurement in postpartum hemorrhage. Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
* Respondent: Whenever there is a case of PPH, we measure the blood collected in the calibrated jars and we also estimate the blood on the pads and the delivery mats.
Section D: Facilitators of Use
* Interviewer: Thank you, ma. Facilitators of use of blood loss measurement in postpartum hemorrhage. Explain facilitators of the use of blood loss measurement in postpartum hemorrhage—who are those overseeing it?
* Respondent: The people that oversee it are the doctors and the nurses.
Section E: Barriers to Blood Loss Measurement
* Interviewer: To the last section: barriers that hinder blood loss measurement in postpartum hemorrhage detection. Describe the barriers against blood loss measurement in postpartum hemorrhage detection in this healthcare facility.
* Respondent: One of the barriers is that the equipment are not readily available and they are not even sufficient.
* Interviewer: What of a high ratio of patients to nurses? Does that too affect the blood loss measurement?
* Respondent: Yes, the high ratio of patients to nurses affects it because we have a lot of workload, so we might not be able to accurately measure the blood loss.
* Interviewer: What of training and retraining? Are the health personnel going for workshops or seminars to have new knowledge about what is in vogue?
* Respondent: I think there is a lack of training and retraining for health workers on new methods of measuring blood loss.
* Interviewer: Some people will be like, "This is what we've been using since," and you can't just come and bring a new thing. Are they ready to accept new innovations?
* Respondent: Yes, we are ready to accept new innovations if provided.
* Interviewer: Mention specific resources or equipment limitations that hinder blood loss measurement.
* Respondent: Lack of calibrated drapes and also lack of enough calibrated jars.
* Interviewer: And the government of these days in Nigeria—are they providing those instruments?
* Respondent: No, the government is not providing them.
* Interviewer: What of—is there any NGO or individual that is providing those?
* Respondent: No.
* Interviewer: Thank you very much, ma.

*Respondent 15
Interview Transcript: Utilization of Blood Loss Measurement Tools
Interviewer: Student Nurse, Ladoke Akintola University of Technology (LAUTECH)
Respondent: Nurse Midwife, Adeoyo Maternity Teaching Hospital (3+ years experience)
Section A: Socio-Demographic Data
* Interviewer: Good morning, ma. How are you doing?
* Respondent: I’m fine. Good morning, ma.
* Interviewer: I am a student nurse from Ladoke Akintola University of Technology, Ogbomoso, a 500-level student. I want you to help me out with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among healthcare workers in Adeoyo Maternity Teaching Hospital, Yemetu, Ibadan."
* Interviewer: So, Section A is the socio-demographic data. Ma, how old are you?
* Respondent: I'm 26.

* Interviewer: Your gender, ma?
* Respondent: Female.
* Interviewer: Religion?
* Respondent: Christian.
* Interviewer: Marital status?
* Respondent: Single.
* Interviewer: Profession? Are you just a nurse or a nurse midwife?
* Respondent: I'm a nurse midwife.
* Interviewer: Years of practice?
* Respondent: Three years plus.

Section B: Availability of Blood Loss Measurement Tools

* Interviewer: Section B: Availability of blood loss measurement tools in postpartum hemorrhage detection. Describe the availability of blood loss measurement tools in postpartum hemorrhage detection in this healthcare facility.
* Respondent: The ideal blood measurement tool is not available in my facility, but most times we use things like pads and gauze to estimate blood loss.
* Interviewer: Kindly explain the tools or equipment available for blood loss measurement.
* Respondent: For cesarean sections, through suctioning, there is always a calibrated jar which you can use to measure. But in normal delivery, we use pads. Most times we use pads to estimate.
* Interviewer: Do you have any blood drapes at all in this facility?
* Respondent: We do not.

Section C: Use of Blood Loss Measurement Tools

* Interviewer: Section C: Use of blood loss measurement tools. Describe how you use them to measure blood loss whenever there is postpartum hemorrhage.
* Respondent: Most times we apply pads. If it is through delivery, we are definitely going to have a head nurse and the person taking the delivery plus an assistant. The assistant is not involved in helping us to measure the blood loss; it is either the head nurse or the person taking the delivery.
* Respondent: So we apply pads. Once it is soaked, that is like 100 mls. Putting another one, if another one is completely soaked, that's another 100 mls. So it goes on like that.
* Interviewer: So when it gets to like 500 mls—like changing the pads like five times within how many hours—will you now say the person is having postpartum hemorrhage?
* Respondent: Within an hour. After delivery, within an hour, it's not supposed to be too much. If at all anything is coming, it should be lochia and it should be gradual.
* Interviewer: Do you know that there is primary hemorrhage, secondary hemorrhage, and tertiary hemorrhage? Primary is within 24 hours. The secondary one is within that six weeks. Have you seen patients come for secondary postpartum hemorrhage?
* Respondent: I've seen a patient come with a secondary postpartum hemorrhage case. According to her, she was sleeping and suddenly she felt something gushing out from her vagina. The diagnosis of secondary postpartum hemorrhage was made.

Section D: Facilitators of Use

* Interviewer: Facilitators of the use of blood loss measurement tools. Who are the facilitators?
* Respondent: Most times the nurses, because during antenatal care we tell them [the patients] things they are to expect during delivery and after delivery so they will cooperate with us. So most times it's always on the nurses.
* Interviewer: What about during cesarean sections?
* Respondent: During cesarean section, it's either the midwife that is to receive the baby or the theater nurse.

Section E: Barriers to Blood Loss Measurement

* Interviewer: Section E: Barriers that hinder blood loss measurement. Describe the barriers in this facility.
* Respondent: One of it is knowledge—adequate knowledge about this particular thing we are talking about. Another one is individual belief or perspective. Some people will not want you to take their blood; they are skeptical about you taking their blood somewhere to measure.
* Respondent: Also cultural belief and religion, because some people will be like "Don't use my blood for this." Another thing is the cooperativeness of the patient. A patient may not want to cooperate; the patient may be using one pad and discarding it without even informing the nurses.
* Interviewer: Mention specific resource or equipment limitations.
* Respondent: Inadequate availability of the materials. Another one is the knowledge that the nurses even need to use in making sure all these things are put into consideration. Inadequate education on the aspect of the patient in question.
* Interviewer: Thank you very much. I actually appreciate it. Thank you for your time.
* Respondent: You're welcome.

*Respondent 16
Interview Transcript: Utilization of Blood Loss Measurement Tools
Interviewer: Student Nurse, Ladoke Akintola University of Technology (LAUTECH)
Respondent: Nurse Midwife, Adeoyo Maternity Teaching Hospital (5 years experience)
Section A: Socio-Demographic Data
* Interviewer: Ma, please how old are you?
* Respondent: I'm 28.
* Interviewer: Gender, ma?
* Respondent: Female.
* Interviewer: Religion?
* Respondent: Christian.
* Interviewer: Marital status?
* Respondent: Married.
* Interviewer: Profession?
* Respondent: Nurse and midwife.
* Interviewer: Years of experience?
* Respondent: Five years.
Section B: Availability of Blood Loss Measurement Tools
* Interviewer: Describe the availability of blood loss measurement tools in postpartum hemorrhage detection in this healthcare facility.
* Respondent: In my current healthcare facility, I think that people are not really informed about blood loss measuring tools, but in my facility, we have one or two tools that we usually use.
* Interviewer: Kindly explain the tools or equipment available for blood loss measurement tools in this your facility.
* Respondent: We have the calibration jar, and then we also use gauze to measure the blood loss, and we also use pads to measure the blood loss.
Section C: Use of Blood Loss Measurement Tools
* Interviewer: Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
* Respondent: One of the things we use is the pad of the patient to measure it. We gather the number of pads the patient uses together, then we weigh them. We first know the weight of the pad before inserting the pad, then after the pad has been used, we weigh it and we are able to know the amount of blood that has been lost through that particular pad. Then we can add everything together to know how much blood has been lost.
Section D: Facilitators of Use

* Interviewer: Explain facilitators of the use of blood loss measurement tools in postpartum hemorrhage detection in this health facility.
* Respondent: Those are the midwives and the nurses. All those involved in care, but majorly the nurses and the midwives.

Section E: Barriers to Blood Loss Measurement

* Interviewer: Describe the barriers against blood loss measurement in postpartum hemorrhage detection.
* Respondent: In most facilities, I think it's lack of the instruments they are meant to use. Some facilities may not have enough measuring tools, and some may not be so informed—so lack of knowledge or information about it, and lack of exposure.
* Interviewer: Mention specific resources or equipment limitations that hinder blood loss in postpartum hemorrhage.
* Respondent: That's the absence of the jars. Also, the absence of power supply. Then when the patients are not able to have enough funds to be able to get the necessary things needed, it can hinder measurement of blood loss. Finally, lack of maintenance; if the equipment available is not well maintained, once it gets spoilt and they are not able to replace it, it won't be available for the next use.

*Respondent 17

Interview Transcript: Utilization of Blood Loss Measurement Tools

Interviewer: Student Nurse, Ladoke Akintola University of Technology (LAUTECH)

Respondent: Nurse Midwife, Adeoyo Maternity Teaching Hospital (5 years experience)

Section A: Socio-Demographic Data

* Interviewer: Good morning, ma. How are you doing?
* Respondent: Fine, thank you.
* Interviewer: I am a student nurse from Ladoke Akintola University of Technology, Ogbomoso, a 500-level student. I want you to help me out with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among healthcare workers in Adeoyo, Yemetu, Ibadan."
* Interviewer: So, it has Section A to E. The first section is socio-demographic data. Ma, please, how old are you?
* Respondent: I’m 27 years old.
* Interviewer: Gender?
* Respondent: Female.
* Interviewer: Religion?
* Respondent: Christianity.
* Interviewer: Marital status?
* Respondent: Single.
* Interviewer: Profession?
* Respondent: Nurse midwife.
* Interviewer: Year of experience?
* Respondent: Five years.

Section B: Availability of Blood Loss Measurement Tools

* Interviewer: Section B: Availability of blood loss measurement tools in postpartum hemorrhage. Describe the availability of the blood loss measurement tools in postpartum hemorrhage detection in this healthcare facility.
* Respondent: In this healthcare facility, so far that I know of, we make use of drapes. We make use of drapes, then we also make use of pads, because some of our patients may not have enough resources like money to purchase some of these things, so at least we try to make sure that they buy one or two things that is not very expensive like pads. We can make use of pads and then underlay too. We make use of underlay as well.

* Interviewer: What of during cesarean section, what do you use?
* Respondent: During cesarean section, we mostly use visual estimation to...
* Interviewer: You don't use suctioning jar? You know suctioning jar is calibrated.
* Respondent: Yes, we... it's not all the time, because of the state of the facility. Sometimes if there is light, we make use of it. But if there is no light, we can't make use of suctioning machine.
* Interviewer: Kindly explain the tools or equipment available for blood loss measurement tools in this facility.
* Respondent: Okay, we have suctioning machines. We use suctioning machines. Then blood drapes—we have blood drapes as well, but it's limited. We don't have a lot, so we don't use for all patients. But we majorly use pads and gauze and underlay.

Section C: Use of Blood Loss Measurement Tools

* Interviewer: To Section C: Use of blood loss measurement in postpartum hemorrhage. Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
* Respondent: Personally, I use... sometimes I use visual estimation depending on the procedure. If it's safe vaginal delivery, we can use visual estimation just to estimate. And then if it's more than 500 mls, we can say the patient is prone to having postpartum hemorrhage. Then for cesarean section, if the blood loss is over 1000 mls, then we can say the patient is at risk of postpartum hemorrhage.

Section D: Facilitators of Use

* Interviewer: Section D: Facilitators of the use of blood loss measurement tools in postpartum hemorrhage. Explain the facilitators of the use of blood loss measurement tools in detection of postpartum hemorrhage.
* Respondent: Sometimes we go on training. They do regular training to enhance and to aware... to make to let there be awareness that we can make use of other things to estimate blood loss. Some of our heads of nurses, they try to say we should make use of the suctioning machine so that we can have adequate measurement of blood loss. And then some people still, because of lack of knowledge and they just want to still do their own practice, so they make use of visual estimation.

Section E: Barriers to Blood Loss Measurement

* Interviewer: What are the barriers that hinder blood loss measurement? Describe the barriers that hinder blood loss measurement in postpartum hemorrhage detection.
* Respondent: For example, use of suctioning machine—if there is no light, we will not be able to make use of suctioning machine. And then sometimes too if the suctioning machine is not in a good condition, we will not be able to make use of suctioning machine. Then blood drapes too—I think it's a little bit expensive at the moment, so we have limited number of patients that say they can afford the use of blood drapes. So most of them, they use gauze and...
* Interviewer: Mention specific resource or equipment limitations that hinder blood loss measurement.
* Respondent: Things like if there is no adequate suctioning machine, then if we don't have measuring jars too. Some of our measuring jars too, they are not in good condition, so we won't be able to use them. But once in a while, we try to make use of the available equipment that we have.
* Interviewer: Thank you very much, ma. I actually appreciate it.
* Respondent: You're welcome.

*Respondent 18

Here is the interview transcript from the audio file:
Interview Transcript
Interviewer: Good afternoon, sir.

Respondent: Good afternoon.
Interviewer: How are you doing, sir?
Respondent: I'm doing fine.
Interviewer: I am a student from Ladoke Akintola University of Technology, a 500-level student. Please, I want you to help me out with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among health workers." So it has Section A to Section E. My Section A goes as so: Socio-demographic data. Please, how old are you?
Respondent: I'm 35.
Interviewer: Your gender?
Respondent: I'm a male.
Interviewer: Religion?
Respondent: Christian.
Interviewer: Marital status?
Respondent: I am married.
Interviewer: Profession?
Respondent: I'm a medical doctor.
Interviewer: Years of experience?
Respondent: Over five years—eight years, basically.
Interviewer: All right. So Section B: Availability of blood loss measurement in postpartum hemorrhage detection. Describe the availability of blood loss measurement tools in postpartum hemorrhage detection.
Respondent: Yes, um, there are some methods available, but I would say they are crude methods; methods that sometimes are not precise enough. We just kind of visually estimate the loss that is available on the ground, commonly in our environment and even in these health facilities, so there's limitation of the right tools to use.
Interviewer: What of during Cesarean section?
Respondent: Even during Cesarean section, yes, the theater sometimes may have a little more, but still, it's not the things we expect to have.
Interviewer: Thank you, sir. Kindly explain the tools or equipment available for the blood loss measurement in these health facilities.
Respondent: So, um, the tools available in this health facility for measurement of blood loss, actually, we do have a measuring jar, but it doesn't almost happen all the time. So there are instances where the jars provided are not calibrated. But even when they are not available, we just have to still go ahead, and so sometimes we end up just estimating based on what we can find around the operation site and the lining.
Interviewer: Thank you, sir. To the next section: Use of blood loss measurement in postpartum hemorrhage. Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
Respondent: So, usually when there's postpartum hemorrhage here, what we do many times—usually because most of them would have had a perineal pad applied after the delivery—so when we notice that the level of the soaking of the perineal pad is quite more than expected, we anticipate that there might be a postpartum hemorrhage. And then, usually, we will still not quantify exactly how much it is because, really, we are supposed to go and take these things and weigh them. But because of the logistic challenges around that time, many times we just assume that, okay, for this part to be soaked, maybe this person must have lost close to maybe 500 or maybe more than that, depending on how many pads are soaked. So that's how we usually do it here.
Interviewer: Thank you, sir. To Section D: Facilitators of blood loss measurement tools in postpartum hemorrhage detection. Explain facilitators of the use of blood loss measurement in postpartum hemorrhage detection in this health facility.

Respondent: Okay, so many times, the people that are usually keen on blood loss facility as in this place, usually the anesthetist on ground, the nurse on the theater—nurse on ground—their ways of looking out for it. Then I would say the whole team, but then there are more people that are more keen on it—the nurses, the anesthetist—they are the ones that usually facilitate the estimation of blood loss here.
Interviewer: Thank you, sir. The last section: Barriers that hinder blood loss measurement in postpartum hemorrhage. Describe the barriers against blood loss measurement in postpartum hemorrhage detection.
Respondent: Typically, because we practice in the third world, there is limitation to what we can do. Usually, ideally, we're supposed to not limit our estimation to just visual type; we're supposed to be able to do other things, use of standard calibration methods, use of drapes that are cut to exact size so that we can measure adequately. But many times we don't get to use these pads, we don't get to have these jars, we don't get to measure or to weigh the gauze or pads. So because we are in a low-income country.
Interviewer: What of—is there any type like any training or retraining that the health workers are going for, maybe workshops, seminars, to improve their knowledge about the one that are in vogue?
Respondent: Yes, I would say there are instances—in fact, regularly, there are seminars that come up on blood loss management in surgery, postpartum hemorrhage itself. So I would say significantly, many personnel have an idea of what to do. The challenges, the tools, the resources needed to make this happen might not be there. Of course, they know they are to measure the gauze, they are to probably check the calibrated jar, but when you're not even using a calibrated jar or there is no weighing scale to measure before and after, there really will be nothing you can do. Or you're not even using a standard gauze; you're using an improvised cut-and-join gauze. So...
Interviewer: Thank you, sir. Is there any cultural barrier or misconception that can also hinder the use of these accurate measurement tools?
Respondent: Yes, this is Africa, and people take the matter of blood—especially for a woman who just delivered—very seriously. So it's not the kind of person you want to just, say for instance, take the gauze and go and measure somewhere or take the pad and go and measure somewhere. If at all you have to do it, it will be right in their presence and they see what you are doing with it. So all these cultural limitations, they actually limit what we can do. Also, to some of them might even say, "Why are you collecting the blood coming from the private area? What do you want to do with it?" Those kind of things. So they help us to—they actually limit what to do or how far we can go with estimation.
Interviewer: Thank you. The last question: Mention specific resource or equipment limitation that hinder blood loss in postpartum hemorrhage.
Respondent: Yes, so the availability of modern tools, even from the basic weighing scale, is not readily available. Calibrated jars—some of them are calibrated, some of them are not—it's not available. Standard gauze that we know is cut 10 by 10 or 15 by 15; they are not available. So many times we just use pads or use towels that have been sterilized. So those things are not available. If they can be made available, it will help with better accuracy when it comes to estimating blood loss.
Interviewer: Thank you very much, I truly appreciate.

*Respondent 19
Interview Transcript
Interviewer: Good afternoon, sir.
Respondent: Good afternoon.
Interviewer: How are you doing, sir?
Respondent: I'm doing fine.

Interviewer: I am a student from Ladoke Akintola University of Technology, a 500-level student. Please, I want you to help me out with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among health workers". So it has Section A to Section E. My Section A goes as so: socio-demographic data. Please, how old are you, sir?
Respondent: I'm 40
Interviewer: Your gender?
Respondent: I'm a male.
Interviewer: Religion?
Respondent: Islam
Interviewer: Marital status?
Respondent: I am married.
Interviewer: Profession?
Respondent: I'm a medical doctor.
Interviewer: Year of experience?
Respondent: Over five years—eight years, basically.
Interviewer: All right. So Section B: Availability of blood loss measurement in postpartum hemorrhage detection. Describe the availability of blood loss measurement tools in postpartum hemorrhage detection.
Respondent: Yes, um, there are some methods available, but I would say they are crude methods; methods that sometimes are not precise enough. We just kind of visually estimate the loss that is available on ground, commonly in our environment and even in these health facilities, so there is limitation of the right tools to use.
Interviewer: What of during Cesarean section?
Respondent: Even during Cesarean section, yes, the theater sometimes may have a little more, but still, it's not the things we expect to have. Thank you, sir.
Interviewer: Kindly explain the tools or equipment available for the blood loss measurement in these health facilities.
Respondent: So, um, the tools available in this health facility for measurement of blood loss, actually, we do have a measuring jar, but it doesn't almost happen all the time. So there are instances where the jars provided, they are not calibrated. But even when they are not available, we just have to still go ahead, and so sometimes we end up just estimating based on what we can find around the operation site and the lining.
Interviewer: Thank you, sir. To the next section: Use of blood loss measurement in postpartum hemorrhage. Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
Respondent: So usually when there's postpartum hemorrhage here, what we do many times—usually because most of them would have had perineal pad applied after the delivery—so when we notice that the level of the soaking of the perineal pad is quite more than expected, we anticipate that there might be a postpartum hemorrhage. And then, usually, we will still not quantify exactly how much it is because, really, we are supposed to go and take these things and weigh them. But because of the logistic challenges around that time, many times we just assume that, okay, for this part to be soaked, maybe this person must have lost close to maybe 500 or maybe more than that, depending on how many parts are soaked. So that's how we usually do it here.
Interviewer: Thank you, sir. To Section D: Facilitators of blood loss measurement tools in postpartum hemorrhage detection. Explain facilitators of the use of blood loss measurement in postpartum hemorrhage detection in this health facility.
Respondent: Okay, so many times, the people that are usually keen on blood loss facility as in this place, usually the anesthetist on ground, the nurse on the theater—nurse on ground—their ways of looking out for it. Then I would say the whole team, but then there are more people that

are more keen on it: the nurses, the anesthetists. They are the ones that usually facilitate the estimation of blood loss here.
Interviewer: Thank you, sir. The last section: Barriers that hinder blood loss measurement in postpartum hemorrhage. Describe the barriers against blood loss measurement in postpartum hemorrhage detection.
Respondent: Typically, because we practice in the third world, yes, there is limitation to what we can do. Usually, ideally, we're supposed to not limit our estimation to just visual type; we're supposed to be able to do other things. Use of standard calibration methods, use of drapes that we can—that are cut to exact size so that we can measure adequately. But many times we don't get to use these pads, we don't get to have these jars, we don't get to measure or to weigh the gauze or pads. So because we are at the low-income country.
Interviewer: What of—is there any type like any training or retraining that the health workers are going for, maybe workshops, seminars, to improve their knowledge about the one that are in vogue?
Respondent: Yes, I would say there are instances—in fact, regularly, there are seminars that come up on blood loss management in surgery, postpartum hemorrhage itself. So I would say significantly, many personnel have an idea of what to do. The challenges, the tools, the resources needed to make this happen might not be there. Of course, they know they are to measure the gauze, they are to probably check the calibrated jar. But when you're not even using a calibrated jar, or there is no weighing scale to measure before and after, there really will be nothing you can do. Or you're not even using a standard gauze; you're using an improvised cut-and-join gauze. So...
Interviewer: Thank you, sir. Is there any cultural barrier or misconception that can also hinder the use of these accurate measurement tools?
Respondent: Yes, this is Africa, and people take the matter of blood—especially for a woman who just delivered—very seriously. So it's not the kind of person you want to just, say for instance, take the gauze and go and measure somewhere or take the pad and go and measure somewhere. If at all you have to do it, it will be right in their presence and they see what you are doing with it. So all these cultural limitations, they actually limit what we can do. Also, to some of them might even say, "Why are you collecting the blood coming from the private area? What do you want to do with it?" Those kind of things. So they help us to—they actually limit what to do or how far we can go with estimation.
Interviewer: Thank you. The last question: Mention specific resource or equipment limitation that hinder blood loss in postpartum hemorrhage.
Respondent: Yes, so the availability of modern tools, even from the basic weighing scale, is not readily available. Calibrated jars—some of them are calibrated, some of them are not—it's not available. Standard gauze that we know is cut 10 by 10 or 15 by 15; they are not available. So many times we just use pads or use towels that have been sterilized. So those things are not available. If they can be made available, it will help with better accuracy when it comes to estimating blood loss.
Interviewer: Thank you very much. I truly appreciate.

*Respondent 20
Here is the transcript of the interview from the audio file:
Interview Transcript
Interviewer: Good afternoon, ma. How are you doing, ma?
Respondent: I'm fine. Good afternoon.
Interviewer: I am a student from Ladoke Akintola University of Technology, a 500-level student. Please, I want you to help me out with my interview. Can I go on, ma?
Respondent: You can proceed.

Interviewer: My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among healthcare workers in Adeoyo Maternity Teaching Hospital, Yemetu, Ibadan." So the sections are Section A to Section E. So Section A is socio-demographic data. Please, ma, how old are you?
Respondent: I'm 40.
Interviewer: Gender?
Respondent: I am female.
Interviewer: Marital status?
Respondent: I'm married.
Interviewer: Profession?
Respondent: I'm a nurse.
Interviewer: Nurse-midwife or just a nurse?
Respondent: I'm a public health nurse.
Interviewer: All right, ma. Years of experience?
Respondent: 12 years and above.
Interviewer: To Section B: Availability of blood loss measurement in postpartum hemorrhage. Describe the availability of blood loss measurement tools in postpartum hemorrhage detection in this facility.
Respondent: To the glory of God, as everything is advancing, we are in a technology age. So, everything—we are also advancing, even when others are not readily available. But we still have some of those measurements that we use in order to control postpartum hemorrhage in this facility.
Interviewer: All right, ma. Kindly explain the tools or equipment available for blood loss measurement in this facility.
Respondent: As far as this facility is concerned, we have—we have drapes, calibrated jars for the estimation, so just like to measure the blood loss.
Interviewer: Do you use gauze and pads to estimate, like weighing before and weighing after to know the amount of blood that has been soaked?
Respondent: Yes, we are graduating to that level as well. So we use—we are going to make—we weigh the pads and the gauze before we use and after. So to know the differences, the weight, and the loss.
Interviewer: All right, ma. Thank you. Section C: Use of blood loss measurement in postpartum hemorrhage detection. Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
Respondent: Actually, any—when any blood loss that we—any measurement above 500 in a normal safe delivery, that one we count it as blood—postpartum hemorrhage. Then anyone that is above 1,000 in a Cesarean section, so we count it as blood loss. And this with the use of the pad and the gauze that we have even weighed before and after we have dipped it into that blood to—to actually to know the actual amount of the loss. So if it is above 500, so we count that one as postpartum hemorrhage in a safe SVD patient. While that of above 1,000, we count it as postpartum hemorrhage in a Cesarean section patient.
Interviewer: All right, ma. Thank you. Section D: Facilitators of the use of blood loss measurement in postpartum hemorrhage. Explain facilitators of the use of blood loss measurement tools in postpartum hemorrhage detection in this facility.
Respondent: The facilitators include the health practitioners, majorly the doctors, the nurses that are in charge, that they have the knowledge of what we are talking about. So they are the major facilitators.
Interviewer: Thank you, ma. To Section E: Barriers that hinder blood loss measurement in postpartum hemorrhage detection. Describe the barriers against blood loss measurement in postpartum hemorrhage detection in this facility.

Respondent: Especially with the new innovations, the major barrier should be attributed to the culture—cultural barrier. Then because some of the patients, this issue of like, "bringing the pad, let us see, to just like weigh"—some people are very rigid. They are very rigid to—rigid to change. So that one is one of the major factors. Then financial constraints. Sometimes to get all those new available measurement tools that is readily available—so due to the cost, some people may not be able to afford it at the point in time. Then in aspect of the health workers, so ability—the tendency to improve upon the present idea and the present knowledge that you have so that you'll be able to just like advance in the new technology. So that's another major barrier to this new tool we are talking all about.
Interviewer: All right. Thank you, ma. Mention specific resources or equipment limitation that hinder blood loss in postpartum hemorrhage.
Respondent: Number one: financial aspect, in terms of money. So because when you don't have enough money, you'll not be able to just like to put some adequate measure in place to prevent blood loss. But when there is enough—when you have enough—so you are financially okay, you'll be able to just like buy the necessary tools to take care of that one. So I think I'm okay with that one.
Interviewer: Thank you very much, I truly appreciate.
Respondent: You're welcome.

*Respondent 21
Based on the audio file provided, here is the transcription of the interview:
Interview Transcript
Interviewer: Good afternoon, ma. How are you doing, ma?
Respondent: I'm fine. Good afternoon.
Interviewer: I am a student from Ladoke Akintola University of Technology, a 500-level student. I want you to help me out with my interview. Can I go on, ma?
Respondent: Yes.
Interviewer: My topic is "Exploration of the utilization of blood loss measurement in postpartum hemorrhage detection among healthcare workers in Adeoyo Maternity." It is in Section A to Section E. The Section A is socio-demographic data. Please, how old are you, ma?
Respondent: 29.
Interviewer: Your gender?
Respondent: Female.
Interviewer: Religion?
Respondent: Christian.
Interviewer: Marital status?
Respondent: Married.
Interviewer: Profession?
Respondent: Nurse.
Interviewer: Years of experience?
Respondent: Below ten years.
Interviewer: To Section B: Availability of blood loss measurement in postpartum hemorrhage. Describe the availability of blood loss measurement in postpartum hemorrhage detection in this facility.
Respondent: It's not really available; the availability is low. Though we use some tools to measure it, for example, gauze, and sometimes if we have the opportunity to suction, we measure the effluent in the suction tube.
Interviewer: All right. Thank you. Kindly explain the tools or equipment available for the blood loss measurement tools in this healthcare facility.
Respondent: Okay. Gauze—we have gauze—the bottle of the suctioning machine, then we have weighing scale to weigh the gauze before and after the use of the gauze.

Interviewer: Do you use drapes at all?
Respondent: Not really. We don't use drapes most of the time.
Interviewer: Thank you, ma. To Section C: Use of blood loss measurement in postpartum hemorrhage detection. Describe how you use to measure blood loss whenever there is hemorrhage.
Respondent: One of the things that we use—we will first observe if there is more blood flow. Then we can try to clean with gauze and the heavier the gauze is, the more we weigh the gauze. Weighing scale, we use gauze, we use suctioning tube also, so we suction. Then we try to see the level of the effluent in the suctioning tube.
Interviewer: All right. Thank you, ma. To Section D: Facilitator of the use of blood loss measurement in postpartum hemorrhage. Explain facilitator of the use of blood loss measurement in postpartum hemorrhage detection in this facility.
Respondent: Number one, it's a policy in the facility for us to be able to detect any hemorrhage. Then we have our leaders: the matrons, the ward manager, the charge nurse on duty. They are part of the facilitators.
Interviewer: What of during Cesarean section?
Respondent: During Cesarean section, the surgeon—they are part—because they actually want to know if there is hemorrhage. The surgeon, the scrub nurse, the midwife there.
Interviewer: To Section E: Barriers that hinder blood loss measurement in postpartum hemorrhage. Describe the barriers against blood loss measurement in postpartum hemorrhage in this facility.
Respondent: Okay. Number one: presence of limited equipment. We have limited equipment to use. Also because of the workload. We have workload. Sometimes it might—workload may not explain. Then we have—let me just say basically: the limited equipment and also the workload of the workers.
Interviewer: What of cultural misconception or cultural barrier? Like you taking someone's pad to somewhere to go and weigh, or even collecting it from them?
Respondent: They feel that you might want to use it for a negative purpose.
Interviewer: So the last question: mention specific resource or equipment limitation that hinder blood loss in postpartum hemorrhage. Like the suctioning machine you are using, is there always adequate power supply to use it?
Respondent: Okay. Sometimes we have power supply, but when there is no power supply, we cannot use the equipment.
Interviewer: Are they actually functioning well?
Respondent: They're functioning well to some extent. There's power supply, then the availability of the equipment. Sometimes we have to share one equipment in many places, so limited equipment is part of it.
Interviewer: What of financial constraint? You know, in some facility they ask the pregnant woman to bring the materials to use, like pads, gauze. Are they always compliant?
Respondent: Not everyone. As you said, financial constraint is also part of it because if they are not able to get the needed materials, they might not be used for them. So, financial constraint on the part of the patients.
Interviewer: Thank you very much, I truly appreciate your time.
Respondent: You're welcome.

*Respondent 22
Here is the transcript of the interview from the audio file:
Interview Transcript
Interviewer: Good afternoon, sir. How are you doing, sir?
Respondent: I'm fine.

Interviewer: I am a student nurse from Ladoke Akintola University of Technology, Ogbomoso, a 500-level student. I want you to help me out with my interview. My topic is "Exploration of the utilization of blood loss measurement tools in postpartum hemorrhage detection among healthcare workers in Adeoyo Maternity Center, Yemetu." So, Section A deals about the socio-demographic data. Sir, please how old are you—your age?
Respondent: I'm 40.
Interviewer: Your gender, sir?
Respondent: I'm male.
Interviewer: Religion?
Respondent: Christian.
Interviewer: Marital status?
Respondent: Married.
Interviewer: Profession?
Respondent: I'm a medical doctor.
Interviewer: Years of experience?
Respondent: Six.
Interviewer: Thank you, sir. To Section B: Availability of blood loss measurement in postpartum hemorrhage detection. Describe the availability of blood loss measurement tools in postpartum hemorrhage detection in this healthcare facility.
Respondent: So we normally use the gauze, the calibrated suction bottle, and visual estimation of the one that falls on the operating table.
Interviewer: For the use of gauze, is it that you already know the weight of the gauze?
Respondent: We always assume that the weight of a small gauze is 10 mls, then Abdo is 250 mls. That's the standardized one we use.
Interviewer: Do you use blood drapes at all?
Respondent: No.
Interviewer: To the next question: kindly explain the tools or equipment available for blood loss measurement in this facility.
Respondent: Again, we always assume that a full soaked gauze is 10 mls, then Abdo pack is 250 mls. Then the calibrated suction jar—we estimate the blood loss, you minus the normal saline, the liquor, or whatever they use to clean (it's either normal saline or liquor). Then we do a visual estimation of the blood on the drape and on the operating table, and just sum it up.
Interviewer: Thank you, sir. Section C: Use of blood loss measurement in postpartum hemorrhage. Describe how you use to measure blood loss whenever there is postpartum hemorrhage.
Respondent: So, in postpartum hemorrhage, after the patient might have been transferred, we always use Abdo, the comfy pad, to estimate the blood loss. And we do a visual estimation of clotted blood that is coming out from the introitus.
Interviewer: Thank you, sir. Facilitators of the use of blood loss measurement in postpartum hemorrhage. Explain facilitators of the use of blood loss measurement in postpartum hemorrhage. Who are those that are overseeing it?
Respondent: So the anesthetist, the nurse, then the obstetricians.
Interviewer: The midwives?
Respondent: And midwives, too.
Interviewer: So to the last section: Barriers that hinder blood loss measurement in postpartum hemorrhage detection. Describe the barrier against blood loss measurement in postpartum hemorrhage detection in this healthcare facility. Is it that the equipment is not readily available—are they adequately available?
Respondent: Actually, the equipment is not readily available because patients—it's out-of-pocket payment. So we have to wait until patients go and buy all those things—the pad and all those things—before you can use them.

Interviewer: What of a high ratio of patients to nurses or doctors? Does that too affect the blood loss measurement?
Respondent: Before, but now the patient ratio has reduced now. So it was before those things affected us.
Interviewer: What of training and retraining—are the health personnel going for workshops, for seminars, to have new knowledge about what is in vogue?
Respondent: I'm not sure there is training and retraining. I'm not sure.
Interviewer: What of—you know some people they will be like, "this is what we've been using since," and you can't just come and bring a new thing. Are they ready to accept new innovations?
Respondent: They are ready to accept new innovations; they are ready if it's worthwhile.
Interviewer: Thank you, sir. So to the last question: mention specific resources or equipment limitations that hinder blood loss measurement in postpartum hemorrhage.
Respondent: Power. Power does not allow us to use the suction machine very well. Then some patients have—they don't really have money, too, so financial constraint. Then sometimes patient ratio to midwife, also.
Interviewer: And the government of these days in Nigeria—are they providing those instruments?
Respondent: No.
Interviewer: What of—is there any NGO or individuals that are providing them?
Respondent: No NGO.
Interviewer: Thank you, sir. I truly appreciate your time.
Respondent: Bye-bye.